

How to Open a Residential Care or Assisted Living Facility in Oregon

A step-by-step guide to the ODHS license — the difference between a residential care facility, an assisted living facility, and a memory care community, the licensed administrator you need, building and staffing your facility, and getting paid

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The Oregon model — read this first

Oregon licenses residential care facilities (RCFs) and assisted living facilities (ALFs) as community-based care. The license comes from the Oregon Department of Human Services (ODHS), Aging and People with Disabilities (APD), under OAR Chapter 411, Division 054 and ORS 443.400 to 443.455. Both provide room, board, and personal care and services on a 24-hour basis in a home-like, person-centered setting — the main difference is the building: an assisted living facility offers apartment-style units with a private bathroom and kitchenette, while a residential care facility offers residential rooms. Either can add a Memory Care Community endorsement (OAR 411-057) for dementia care. Two things matter more than anything else when you start: your administrator must hold an Oregon Residential Care Facility Administrator license, and you must have written confirmation of licensure from ODHS before a single resident moves in. This guide walks you through the license, the building, the administrator, staffing, residents, and how you get paid — privately and through Oregon Medicaid.

How to Use This Guide

This guide takes you from idea to an operating Oregon residential care or assisted living facility. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Oregon facility owners — not legal, tax, or licensing advice. Requirements, fees, and forms change, so verify each step with the agency named before you rely on it. Where this guide gives a figure, treat it as a starting point to confirm, not a final answer.

The one-paragraph version

Decide your model — a residential care facility (rooms), an assisted living facility (apartments), and whether to add a memory care community endorsement. Register the business with the Oregon Secretary of State and secure a licensed Residential Care Facility Administrator. If you are building or converting, work with ODHS Facilities, Planning and Safety before you pull a building permit. Submit your complete license application (SE 0570) to ODHS at least sixty days before you expect to open, with your policies and procedures, proof of fiscal responsibility and a twelve-month pro forma, your residency agreement and disclosures, your caregiver training protocol, and the name of the nurse overseeing resident health services. Run background checks through your Qualified Entity Designee, pass the licensing survey, and get written confirmation of licensure from ODHS — then, and only then, move residents in. Serve residents privately and, if you contract with ODHS, through Oregon Medicaid.

The Path at a Glance

Ten steps take you from idea to an operating Oregon residential care or assisted living facility:

- 1. Choose your model.** Residential care facility (rooms), assisted living facility (apartments), and whether to add a memory care community endorsement.

2. **Form the business.** Register the facility name with the Oregon Secretary of State; get an EIN; open a business bank account.
3. **Secure a licensed administrator.** A full-time Oregon Residential Care Facility Administrator, on-site at least forty hours a week.
4. **Plan the building.** For new construction or conversion, work with ODHS Facilities, Planning and Safety before you pull a building permit.
5. **Write your policies.** Adopt the policy and procedure manual, resident forms, and disclosures built to OAR Chapter 411, Division 054.
6. **Apply to ODHS.** Submit your complete license application (SE 0570) at least sixty days before expected licensure.
7. **Prove you can operate.** Fiscal responsibility and a twelve-month pro forma, insurance, a nurse for resident health services, and your staffing plan.
8. **Screen and train staff.** Background checks through your Qualified Entity Designee, caregiver training, competency, and dementia training.
9. **Pass the survey and get written licensure.** Get written confirmation of licensure from ODHS — before any resident moves in.
10. **Admit residents and grow.** Serve residents on a residency agreement and person-centered service plan — private pay and, by contract, Oregon Medicaid.

Part 1 — What This Business Is

A residential care or assisted living facility gives older adults and people with disabilities a place to live where room, board, personal care, and services are available around the clock — help with bathing, dressing, grooming, and medications; meals and housekeeping; activities and social life; and coordination of health care — in a home-like setting that emphasizes choice, dignity, privacy, and independence. In Oregon this is community-based care, licensed by ODHS, Aging and People with Disabilities, and it is the model this guide is built around.

Oregon has strong demand for community-based care: a growing older population, a state long-term-care system that deliberately favors home-like settings over institutions, and Medicaid funding that follows residents into licensed facilities. The trade-off is real regulatory structure — a building that meets code, a state-licensed administrator, trained and screened staff, and an ODHS survey — but meeting those standards is exactly what lets families, hospitals, and case managers trust you with a loved one.

It helps to be clear-eyed about what kind of business this is. Unlike in-home care, where you can start small and light, a residential care or assisted living facility is a real-estate and operations business first: you are responsible for a building, around-the-clock staffing, meals, and the safety of people who live under your roof. That makes it more capital-intensive and slower to open — but it also builds a durable asset and a steady, occupancy-based revenue stream once you are established. Deciding that this is the business you want, with both its weight and its rewards, is the real first step.

The three models, in plain terms

- **Residential care facility (RCF):** residents live in furnished or unfurnished rooms and receive care and services; the most flexible and common way to start, from small homes to larger buildings.
- **Assisted living facility (ALF):** residents live in apartment-style units — each with a lockable door, a private bathroom, and a kitchenette — with care and services available; a more independent, apartment model.
- **Memory care community:** an endorsement added to an RCF or ALF to serve residents with Alzheimer's or other dementia, with dementia-trained staff and a secure, purpose-built environment.

Part 2 — The Oregon License: RCF, ALF & Memory Care

The credential you need is a community-based care license from ODHS, Aging and People with Disabilities, under OAR Chapter 411, Division 054 and ORS 443.400 to 443.455. You choose the license type that matches your building and model.

Choosing between a residential care facility and an assisted living facility is really a decision about the building and the resident experience you want to offer — rooms in a shared home versus private apartments — and it has cost, timeline, and marketing consequences, so make it deliberately and early. The memory care endorsement is a separate decision layered on top: it lets you serve residents with dementia and reach a large and growing population, but it adds staffing, training, environment, and disclosure requirements. Many operators start with one clear model and add the endorsement or a second license later.

- **Residential care facility license:** for a facility built to the residential care building requirements (OAR 411-054-0200) — residential rooms rather than full apartments.
- **Assisted living facility license:** for a facility built to the assisted living building requirements (OAR 411-054-0300) — apartment units, each with a lockable lever-handle door, a private bathroom, and a kitchenette.
- **Memory care community endorsement:** added to either license under OAR 411-057 to advertise and provide dementia care, with dementia-specific staffing, training, environment, and disclosures.

The rule that trips people up: license before move-in

You may not move a single resident into the facility until you have received written confirmation of licensure from the ODHS Community-Based Care Licensing Unit. Opening the doors early — even to one resident, even briefly — is

operating without a license. Build your timeline around getting that written confirmation first, and never let a move-in date get ahead of it.

Part 3 — New Construction & the Building

Because this is a facility, the building itself is part of the license — and Oregon reviews it before you open. If you are building new, converting another building, or making major alterations, start with the building long before you plan to admit residents.

- **Start with Facilities, Planning and Safety (FPS).** Before you apply for a building permit, prospective RCF and ALF applicants must follow the new-construction process in OAR 411-054-0012 and work with ODHS Facilities, Planning and Safety.
- **Build to Oregon's specialty codes.** Your facility must meet the Oregon Structural, Mechanical, Electrical, and Plumbing Specialty Codes, plus the RCF or ALF building requirements in Division 054.
- **ALF apartments have specific rules.** Each assisted living unit must have a lockable entry door with a lever handle, a private bathroom, and a kitchenette — adaptable units are not acceptable — and a multi-floor building needs an elevator.
- **Finish before you license.** Send the Project Substantial Completion Notice to FPS about thirty days before your anticipated licensure so the building is ready for the licensing survey.

The building is the long pole in the tent

Construction, code review, and FPS sign-off take longer than any paperwork, so they set your real timeline. Engage FPS early, keep your architect and contractor working to the Division 054 requirements from the first drawings, and don't order furniture or set a move-in date until the building is on track — changing a building after the fact is far more expensive than getting the drawings right.

Part 4 — The Licensed Administrator

Every Oregon residential care and assisted living facility must employ a full-time administrator who is scheduled on-site at least forty hours a week — and, since January 1, 2022, that administrator must hold an Oregon license.

- **The Residential Care Facility Administrator (RCFA) license.** The administrator must hold an RCFA license from the Oregon Health Authority, Health Licensing Office, under OAR Chapter 853 and ORS 678.710 to 678.820 — which requires meeting education and experience standards, completing the required administrator training, and passing the Oregon laws and rules examination.
- **It is a real, ongoing license.** The administrator maintains the license through annual continuing education and the standards of practice and professional conduct set by the Long Term Care Administrators Board.
- **Secure yours early.** A facility cannot be licensed or operate without a qualified administrator, so line up your licensed administrator before you apply — whether that is you (if you become licensed) or someone you hire.

If your administrator ever leaves, you must notify ODHS immediately and arrange a qualified interim administrator — so plan for coverage, not just for hiring.

It is worth understanding why Oregon treats the administrator role so seriously. The administrator sets the tone for resident care, staffing, and compliance, and the state holds that person personally accountable through the license. For an owner, this has a practical consequence: either you invest the time to become a licensed administrator yourself, or you budget for a qualified one as a key hire and give them the authority to actually run the building. Trying to operate on the cheap here — an absent or underqualified administrator — is one of the fastest ways to a survey problem.

Part 5 — Applying to ODHS

Once your model, building, and administrator are lined up, you apply to ODHS. The application is thorough because ODHS is deciding whether you can safely house and care for vulnerable adults. Submit your complete license application (SE 0570) at least sixty days before you expect to be licensed; if one entity owns the facility and another operates it, the operator files a management application (APD 0570M) too.

Give yourself more than the minimum sixty days if you can. The application draws together your building, your administrator, your finances, your policies, and your staffing all at once, and gaps in any one of them slow the whole thing down. Treat the sixty-day mark as the latest you should file, not the target, and use the extra time to make sure every piece is complete and consistent before it reaches ODHS.

What you submit

- **Ownership and fiscal responsibility.** Each owner of ten percent or more (five percent if you serve Medicaid) is part of the application; you show proof of fiscal responsibility and a twelve-month pro forma of revenues, expenditures, and resident days.
- **Your policies, disclosures, and agreements.** Your policy and procedure manual, your written residency agreement, and your Uniform Disclosure Statement (and the memory care version if endorsed).
- **Your care and staffing systems.** Your caregiver training protocol, your resident evaluation and service-planning tools, your staffing plan, and the name of the nurse who will oversee resident health services.
- **Background checks.** Your Qualified Entity Designee is approved to submit background-check requests for owners, the administrator, and staff.
- **Administrator information.** The Administrator Reference Summary about thirty days before licensure.

A note for first-time operators

If you have never managed an RCF or ALF and you are converting an existing building, Oregon requires you to retain a consultant with residential-care experience for the first six months. Budget for that help — and treat it as an asset, not just a cost, because an experienced consultant will keep your first survey and your first months out of trouble.

Part 6 — Staffing, Training & Background Checks

Your residents are only as safe as your staff, and Oregon expects you to screen, train, and verify every caregiver.

Staffing is where a facility's quality is won or lost, so build your team with the same care you build your building. Recruiting, screening, training, and retaining good caregivers is an ongoing job, not a one-time hire before opening — turnover is the norm in this field, and a thin or poorly trained team shows up immediately in resident care and in survey findings. Plan for a training pipeline, competency checks on a schedule, and enough staff that no shift is left short, and treat your caregivers as the people who actually deliver the service families are paying for.

- **Caregivers and universal workers.** Direct care staff — or universal workers who cross roles — provide personal care under your training program, matched to the services you offer.
- **Background checks first.** Complete Oregon background checks through your Qualified Entity Designee before staff provide care.
- **Train and verify competency.** Follow your caregiver training protocol and verify competency by observation and written or verbal testing before staff work independently — not just at hire, but on a schedule.
- **Dementia training for memory care.** If you are endorsed as a memory care community, staff complete approved dementia training and person-centered dementia care before providing care.
- **Enough staff, awake and available.** Staff your facility to residents' actual needs around the clock — your staffing plan is part of your license.

Part 7 — Residents: Admission, Rights & the Service Plan

How you admit and care for residents is the heart of the rules. Get this right and most of the survey takes care of itself.

Everything in this part comes back to one idea: you admit only residents whose needs you can actually meet, you plan their care with them, and you document it. Surveyors look hard at the match between what a resident needs and what your facility is licensed and staffed to provide, at whether the service plan reflects the resident's own goals and choices, and at how you respond when a resident's condition changes. Build those habits from your first resident and they become simply how your facility runs.

- **Disclosure and the residency agreement.** Give every prospective resident your Uniform Disclosure Statement and a written residency agreement that spells out services, rates, and rights before move-in.
- **Evaluation at move-in.** Evaluate each resident's needs and preferences at move-in, and only admit residents whose needs you are licensed and able to meet.
- **The person-centered service plan.** Build a person-centered service plan for each resident; for a Medicaid resident, the APD or Area Agency on Aging case manager (the person-centered service plan coordinator) leads it, and you carry it out.
- **Resident rights.** Honor residents' rights to dignity, choice, privacy, and to voice grievances without reprisal — and follow the rules on change of condition, health services, medications, and involuntary move-out.
- **Protect residents and report abuse.** Prevent abuse, and report and investigate any suspected abuse under OAR 411-054-0028 and ORS 124; call 911 in an emergency.

Part 8 — Getting Paid

Residential care and assisted living are paid two main ways: privately by residents and families, and by Oregon Medicaid for eligible residents. Most facilities serve a mix.

Your payer mix is one of the most consequential early decisions you make, because it shapes your rates, your paperwork, and even your location. It is worth modeling a few scenarios — mostly private pay, mostly Medicaid, and a blend — against your real costs before you commit, so you open with a clear-eyed view of how the facility actually makes money.

Private pay

Set clear rates in your residency agreement — a base rate for room and board plus the services each resident needs — and disclose them up front in your Uniform Disclosure Statement. Private pay needs no Medicaid contract, and rates in the Portland metro tend to run higher than in rural Oregon. Build your real costs — staffing around the clock, the licensed administrator, the nurse, food, insurance, and debt service on the building — into your rates.

Oregon Medicaid (community-based care)

To serve Medicaid residents, you contract with ODHS to provide community-based care. For a Medicaid resident, the APD or Area Agency on Aging case manager completes the person-centered service plan and authorizes services, and ODHS pays the facility under its rate structure while the resident pays their share toward room and board. Serving Medicaid opens a much larger population and steady referrals from case managers — the trade-off is contracting, the five-percent ownership-disclosure threshold, and billing to the state's rules.

Build your pro forma on real numbers

Your twelve-month pro forma is not just an application form — it is the plan that keeps you solvent. Base it on realistic occupancy that ramps up over months (not a full building on day one), your true staffing costs, and your actual private-pay and Medicaid mix. New facilities most often get into trouble by assuming they will fill quickly; plan for a slower ramp and fund the gap.

Occupancy is the number that determines whether a facility thrives or struggles, so treat filling beds as a core operating discipline, not an afterthought. Relationships with hospital discharge planners, case managers, and senior-placement advisors are what keep referrals coming, and they take months to build — which is another reason to start that outreach the moment you are licensed rather than waiting until beds sit empty.

Part 9 — The Licensing Survey & Your First 90 Days

1. **Before you apply:** Choose your model, form the business, secure your licensed administrator, and engage FPS on the building.
2. **Before you apply:** Adopt your policies, residency agreement, disclosures, training protocol, and service-planning tools to Division 054, and line up your nurse and insurance.
3. **Apply:** Submit SE 0570 at least sixty days out, with your fiscal-responsibility proof and pro forma, and set up background checks through your QED.
4. **Before licensure:** Finish the building, send the Project Substantial Completion Notice to FPS, hire and train staff, and prepare for the on-site licensing survey.
5. **After written licensure:** Only now admit your first residents — on a residency agreement and person-centered service plan — and begin outreach to hospital discharge planners, case managers, and senior referral sources.

Part 10 — Common Mistakes to Avoid

- **Moving a resident in before written licensure.** This is the single most serious mistake — wait for ODHS's written confirmation.
- **Underestimating the building.** FPS review and construction set your timeline; engage them before the building permit, not after.
- **Not lining up a licensed administrator early.** You cannot license or operate without a qualified RCFA — secure yours before you apply.
- **Confusing RCF and ALF requirements.** Apartments with kitchenettes are an ALF requirement; build to the model you are licensing.
- **A pro forma built on full occupancy.** Plan for a months-long ramp and fund the gap, or you will run short of cash.
- **Skipping the six-month consultant.** If you're a first-time operator converting a building, Oregon requires it — and you'll want it.
- **Weak background checks or training records.** Screen through your QED and document training and competency before staff work independently.

Part 11 — Staying Licensed & Growing

Once you are open, compliance is ongoing: renew your license on schedule; keep your administrator's license, your staff background checks, training, and competency current; keep residency agreements, disclosures, and person-centered service plans updated; maintain fire and life safety and your emergency and disaster plan; and participate in the ODHS Quality Measurement Program, which tracks quality across Oregon community-based care. Keep a compliance calendar so nothing lapses, and stay ready for an ODHS inspection or complaint investigation at any time.

The operators who stay out of trouble treat compliance as a daily operating habit rather than a scramble before a survey. Your administrator, your nurse, and your lead caregivers should share a rhythm of checking the things that matter — are service plans current, are staff trained and screened, are medications and change-of-condition responses documented, is the building safe — so that an unannounced inspection finds a facility that is already in order. That rhythm is also simply good care, which is the point.

Growth comes from three moves: build referral relationships — with hospital discharge planners, APD and Area Agency on Aging case managers, and senior-placement advisors — until you are the facility they recommend; add payers and services (layer Medicaid onto private pay, and consider a memory care endorsement to serve more residents); and expand capacity thoughtfully — more beds or a second location — at a pace your administrator,

staffing, and building can actually support. As you grow, protect the things a survey looks at hardest: your staffing levels, your service plans, and your abuse-prevention and reporting.

Part 12 — What It Costs to Start

Residential care and assisted living are more capital-intensive than in-home care because you are licensing a building. Plan for the following, and gather real quotes rather than guessing:

- **The building.** Construction, purchase, or conversion to Division 054 standards is by far the largest cost — and the one to plan first.
- **ODHS license fee (by licensed beds).** For a new facility license: about \$2,000 for 1–15 beds, \$3,000 for 16–49, \$4,000 for 50–99, \$5,000 for 100–150, and \$6,000 for 151 or more — plus a memory care fee (roughly \$50 to \$100) if endorsed. Confirm current amounts with ODHS; a separate invoice is sent.
- **The licensed administrator.** A full-time RCFA on-site at least forty hours a week — a core ongoing cost.
- **Nursing and staffing.** A nurse to oversee resident health services, plus caregivers staffed around the clock — your largest recurring expense.
- **Insurance and business setup.** General and professional liability and other coverage; Secretary of State registration and your accounting setup.
- **Policies, disclosures, and systems.** Your policy and procedure manual, residency agreement, disclosures, and your background-check, training, and documentation systems.
- **The six-month consultant (if applicable).** For a first-time operator converting a building.
- **Working capital.** Enough cash to carry the building, the administrator, the nurse, and staff through a months-long lease-up before the facility fills — the cost new owners most often underestimate.

Two Worked Examples

Example 1 — A small residential care facility

You want to start smaller, so you open a residential care facility. You secure a licensed administrator, form the business, and work with FPS to bring a residential building up to the Division 054 residential care requirements. You submit SE 0570 more than sixty days out with your policies, residency agreement, disclosures, fiscal-responsibility proof, and a twelve-month pro forma built on a gradual lease-up. You run background checks through your QED, train your caregivers, pass the licensing survey, and receive written confirmation of licensure. Only then do you admit your first residents — on residency agreements and person-centered service plans — and you begin taking referrals from hospital discharge planners and APD case managers, serving a mix of private-pay and Medicaid residents.

The lesson of this example is that starting with a residential care facility keeps your building and capital requirements more manageable while you learn to operate — screening, service planning, staffing, and surviving your first survey — on a smaller scale. Many successful operators start here and expand later, rather than taking on a large purpose-built facility as a first project.

Example 2 — An assisted living facility with memory care

You are building purpose-built apartments, so you license an assisted living facility and add a memory care community endorsement. You engage FPS before the building permit and build apartment units — each with a lockable lever-handle door, a private bathroom, and a kitchenette — plus a secure, purpose-built memory care area. You license your administrator, hire and dementia-train your staff, and prepare your memory care disclosures. You submit your application and, after the building is substantially complete and the survey is passed, receive written licensure. You open the assisted living apartments and the memory care community, serving residents privately and, under your ODHS contract, through Oregon Medicaid — with the APD or Area Agency on Aging case manager leading each Medicaid resident's service plan.

Who to Call & What to Ask

For...	Who to contact
Forming your business	Oregon Secretary of State, Corporation Division (sos.oregon.gov). Ask: how do I register my facility name and entity, and what are the ongoing requirements?
Licensing your facility	ODHS, Aging & People with Disabilities — Community-Based Care Licensing Unit / Safety, Oversight & Quality Unit (oregon.gov/odhs). Ask: what does a complete SE 0570 application require, and what are the current fees and timeframes?
Building & construction	ODHS Facilities, Planning and Safety (FPS). Ask: what is the new-construction process under OAR 411-054-0012, and what do I submit before a building permit?
Your administrator's license	Oregon Health Authority, Health Licensing Office — Residential Care Facility Administrator license, OAR Chapter 853 (oregon.gov/oha). Ask: how do I qualify for and maintain the RCFA license?
Background checks	ODHS Background Check Unit, through your Qualified Entity Designee. Ask: how do I set up my QED and submit background-check requests?
Serving Medicaid residents	Your local APD office and Area Agency on Aging (AAA). Ask: how do I contract with ODHS to serve Medicaid residents, and how do case managers do service plans?
Fire & life safety	Your local fire marshal and building department, with the Oregon specialty codes. Ask: what fire, life-safety, and code approvals does my facility need?
Reporting abuse	The local APD or Area Agency on Aging office and Adult Protective Services under ORS 124; 911 for emergencies.

Key Terms

- **Community-based care:** Oregon's model for RCFs, ALFs, and memory care communities — home-like, person-centered settings licensed by ODHS/APD.
- **Residential care facility (RCF):** a licensed facility where residents live in rooms and receive care and services.
- **Assisted living facility (ALF):** a licensed facility with apartment units — lockable door, private bathroom, and kitchenette.
- **Memory care community:** an endorsement (OAR 411-057) to serve residents with dementia, with dementia-trained staff and a secure environment.
- **RCFA license:** the Oregon Residential Care Facility Administrator license, required to run an RCF or ALF, from the OHA Health Licensing Office.
- **FPS:** ODHS Facilities, Planning and Safety — reviews new construction and building plans.
- **Person-centered service plan:** each resident's plan of supports and services; for Medicaid residents, led by the APD or AAA case manager.
- **Uniform Disclosure Statement:** the standard disclosure you give prospective residents before move-in.
- **QED:** Qualified Entity Designee — the person approved to submit background-check requests.
- **Written confirmation of licensure:** ODHS's written approval — required before any resident moves in.

Core Rules & References

- OAR Chapter 411, Division 054 — Residential Care and Assisted Living Facilities (ODHS, Aging and People with Disabilities)
- ORS 443.400 to 443.455 — residential care and assisted living facilities (the governing statutes)
- OAR Chapter 411, Division 057; ORS 443.886 — endorsed Memory Care Community
- OAR Chapter 853; ORS 678.710 to 678.820 — Residential Care Facility Administrator licensing (OHA Health Licensing Office; Long Term Care Administrators Board)
- OAR 411-054-0012 & building requirements (RCF 411-054-0200; ALF 411-054-0300); Oregon specialty codes (structural, mechanical, electrical, plumbing)
- OAR 411-054-0028 and ORS 124 — abuse reporting and investigation; OAR 411-054-0320 — Quality Measurement Program

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